



BOTSWANA NATIONAL HEALTH QUALITY STANDARDS FOR HOSPITALS

41. Psychiatric Electro-Convulsive Therapy Care

NAME OF HEALTHCARE FACILITY: _____

TYPE OF SURVEY: _____

DATE OF SURVEY: _____

DOCUMENT COMPLETED BY (Title and Name)

INTERVIEWEE (Name and Designation)

This Standard Assessment Manual (SAM) provides a means to record the level of compliance achieved for each criterion (NA NC PC C), comments explaining why compliance has not been achieved and actions required to achieve compliance. Each criterion has a guideline statement which provides clarification of the requirements for compliance.

In summary, criteria are scored as follows:

- 1. Compliant (C)** means the condition required is substantially met. Evidence of compliance should be present in a tangible and/or observable form, e.g. documentation, physical items, etc.
- 2. Partially Compliant (PC)** means the condition required is not totally met, but there is progress towards compliance.
- 3. Non-compliant (NC)** means there is no observable progress or minimal progress towards complying with the requirement specified in the criterion.
- 4. Not applicable (NA)** means the criterion will not be scored because the service is either not provided, or not required at the particular level of care the criterion is designed to measure.

Please circle the compliance rating assigned to each criterion. Explanatory comments should be provided for each criterion which does not receive a Compliant (C) score.

Documents Checked

Surveyor:

Surveyor:



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Overview of Psychiatric Electro-Convulsive Therapy Care

Electro-convulsive therapy carries a high risk. The collaboration between personnel in the therapy unit, health and safety personnel and those responsible for the supply and maintenance of equipment is essential.

Professional guidelines must be available to personnel, and closely followed.

The organisation ensures that an adequate number of suitably qualified and experienced personnel are available at all times to provide for a safe electro-convulsive therapy service.

Electro-convulsive therapy is a non-invasive procedure, which is carried out under general anaesthesia. A full-scale operating theatre is not used and everything used, except for injection equipment, is "surgically clean" rather than "sterile".



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Standards

41.1 Management of the Electro-Convulsive Therapy Service

41.1.1 Standard

The electro-convulsive therapy service is managed and staffed to provide a safe and effective service.

Intent of 41.1.1

Electro-convulsive therapy personnel work with organisation leaders to ensure adequate and suitable management processes and staffing of the unit.

The qualifications of those persons who administer anaesthesia in the organisation are documented in accordance with current local/international professional society standards.

41.1.1 Criteria

41.1.1.1	A senior professional who is suitably qualified and experienced oversees the electro-convulsive therapy (ECT) service.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: The job description and qualifications will be reviewed during the personnel record audit.		

41.1.1.2	Electro-convulsive therapy rosters ensure that registered nurses with suitable qualifications and experience are present at all shifts for electro-convulsive therapy unit duties.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Evidence is required in the form of personnel allocation lists, duty rosters and off-duty lists.		

41.1.1.3	Anaesthesia is administered only by qualified anaesthetists.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Proof of current registration with the relevant authority must be available. Compliance will be verified during the patient record and personnel file audit.		

41.1.1.4	The anaesthetist is directly responsible for only one anaesthetic at a time.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: The theatre register will be consulted to ensure that the medical professional responsible for administering anaesthesia is not responsible for the care of more than one patient at a time.		



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41.1.1.5	The medical practitioner performing the procedures is available before the anaesthesia commences.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Personnel interviews and patient record audits must confirm compliance.		

41.1.1.6	Nursing personnel, who are trained and experienced in the administration of electro-convulsive therapy are available until the patient has fully recovered.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Evidence is required in the form of personnel allocation lists, duty rosters and off-duty lists. Proof of current registration with the relevant authority must be available. Compliance will be verified during the patient record and personnel file audit.		

41.1.2 Standard

Clinical practice guidelines are used to guide patient care and reduce unwanted variation.

Intent of 41.1.2

Clinical practice guidelines provide a means for improving quality and assist practitioners in making clinical decisions. Consideration should be given to providing guidelines for high risk, high volume and high cost conditions as these are the areas that represent the highest risk to patients and the organisation. In addition, guidelines should be available for conditions which are rarely seen but may have severe consequences for patients if misdiagnosed or mismanaged.

Guidelines are found in the literature under many names including practice parameters, practice guidelines, patient care protocols, standards of practice, care pathways, etc. Regardless of the source, the scientific basis of guidelines should be reviewed and approved by the clinical leaders and clinical practitioners before implementation. This will ensure that they meet the criteria established by the leaders and are adapted to the community, patient needs and organisation resources. Once adopted, guidelines should be reviewed on a regular basis to ensure their continued relevance.

41.1.2 Criteria

41.1.2.1	Evidence-based clinical practice guidelines relevant to the patients and services of the organisation are available to guide patient care processes.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Sometimes such evidence is available for nursing practices only, in which case this will be scored PC. In the absence of guidelines this criterion will be scored NC.		



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41.1.2.2	Organisational and clinical leaders set criteria to select clinical practice guidelines.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Every effort should be made to identify and adopt guidelines that have been rigorously and scientifically developed, internationally accepted and adapted for local use.		

41.1.2.3	The implementation of guidelines is monitored as part of a structured clinical audit.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Clinical audits cannot be done on ALL guidelines, but evidence must be available that the management of high cost, high risk, high volume and problem-prone conditions are considered for auditing. If guidelines are available but structured clinical audits are not done, this criterion is scored NC. Sometimes such evidence exists for nursing practices only, in which case this will be scored PC.		

41.1.2.4	Guidelines are reviewed on a regular basis and updated when necessary.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Organisational policy should define the frequency with which guidelines are to be reviewed to ensure they reflect current practice. This should include updating of guidelines following major changes in best practice advice as well as routine review.		



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41.2 Assessment of Patients

41.2.1 Standard

All patients cared for by the organisation have their healthcare needs identified through an established assessment process within appropriate time frames.

Intent of 41.2.1

When a patient enters a department, the specific information required and the procedures for obtaining and documenting it depend on the patient's needs and on the setting in which care is being provided.

Certain patients may require a modified assessment, for example, very young patients, the frail or elderly, those terminally ill or in pain, patients suspected of drug and/or alcohol dependence and victims of abuse and neglect. When appropriate, the assessment process should be modified to respect local cultural practices.

The organisation defines, in writing, the scope and content of assessments to be performed by each clinical discipline within its scope of practice and applicable laws and regulations.

These findings are used throughout the care process to evaluate patient progress and understand the need for reassessment. It is essential that assessments are documented well and can be easily retrieved from the patient's record.

The healthcare organisation determines the time frame for completing assessments. This may vary in the different settings within the organisation.

When the medical assessment was conducted by a different healthcare organisation, a legible copy of the findings should be filed in the patient's record. Any significant changes in the patient's condition since this assessment should be recorded.

41.2.1 Criteria

41.2.1.1	Written policies and procedures for assessing patients and their families on admission and during ongoing care are implemented.	Comment:
ROOT		
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: A policy must be available which defines the scope of practice for each discipline, together with specific information required, procedures to be carried out and all documentation to be completed during the assessment of patients and their families. The provision of specifically designed assessment forms for each relevant discipline is considered as evidence of the implementation of such a policy. Where the required information, according to the patient's presenting complaints/condition is not documented, the score will be adjusted accordingly. Compliance will be verified during the patient record audit.		

41.2.1.2	Only those individuals permitted by applicable laws, regulations and professional registration perform the assessments.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit, which will focus on the legibility of signatures and the identification of designations/qualifications. The absence of notes made by the doctor in the patient record, or other relevant clinical personnel other than nursing personnel will affect the outcome of the audit process.		



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41.2.1.3	The scope and content of assessment by each discipline is defined.	Comment:
ROOT		
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: It is accepted that the scope and content of assessment for each discipline is defined if the organisation makes use of standardised formats of clinical assessment forms for specific use by different disciplines. The absence of medical notes or other relevant clinical personnel other than nursing personnel in the patient record will affect the outcome of the audit process. Compliance will be verified during the patient record audit.		

41.2.1.4	Assessments are performed within appropriate time frames and are comprehensively documented in the patient's records according to organisation policy.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Time frames for the assessment of patients should be established for each discipline for example, there should be an indication of which cases are urgent/or not priority cases. It is important to note that this criterion does not refer to the length of time that it takes to perform the assessment, but the time within which the initial assessment should be commenced and/or completed. These time frames should be stated in the policy framework. Compliance will be verified during the patient record audit.		

41.2.1.5	The date and time of assessment can be identified.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

41.2.1.6	The organisation identifies those patient populations and special situations for which the assessment process is modified.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Documented evidence in the form of a detailed policy and procedure or guideline is provided. To ensure coordination of patient care, all members of the team should be included in the formulation of this policy framework, especially doctors and nurses. Compliance will be verified during the patient record audit.		

41.2.1.7	Those special patient populations receive individualised assessments.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		



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41.2.1.8	The findings of assessments performed outside the organisation are verified on admission.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

41.2.1.9	Any significant changes in the patient's condition since the assessment performed outside of the organisation are noted in the patient's record.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

41.2.2 Standard

ECT specific policies and procedures are implemented.

Intent of 41.2.2

Guidelines of professional societies and associations are available and followed wherever anaesthesia is administered. This includes nursing personnel who assist the anaesthetist and who monitor the recovery of patients. Implementing these guidelines is particularly important with regard to the qualifications, training and experience required by personnel members in the service, and also the provision, maintenance and use of medical equipment and drugs.

Controlling bodies also develop guidelines and regulations relating to professional practice. Policies and procedures are necessary to guide the administration of the ECT service to ensure the smooth operation of the service, and to ensure that personnel act swiftly and in a coordinated manner in an emergency. These are made available to all unit, recovery room and anaesthetic personnel, and are known and implemented. Biohazards which need to be monitored and notified include electrical hazards.

41.2.2 Criteria

41.2.2.1	Written policies and procedures guide the activities of the ECT treatment room.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: As a minimum, policies and procedures relating to the following are required for compliance with this criterion: • The duties of ECT treatment room nursing personnel • ECT treatment room cleaning • Notification of biohazards • Medication control • Patient positioning		

41.2.2.2	Policies and procedures are implemented in relation to the preparation of patients for electro-convulsive therapy.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		



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Guideline statement: As a minimum, policies and procedures relating to the following are required for compliance with this criterion:

- Scheduling of patients for procedures
- Informed consent
- Patient identification
- Verification of the treatment
- Verification of the last oral intake
- Equipment required
- Documented of processes during care to ensure coordination and safety

41.2.2.3	Policies and procedures are implemented in relation to the anaesthetic service.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: As a minimum, policies and procedures relating to the following are required for compliance with this criterion:

- Qualifications of persons who administer anaesthetics
- Anaesthetic equipment hazards
- Assessing the fitness of patients to leave the recovery area

41.2.2.4	Policies and procedures comply with current guidelines of the local/international professional society of anaesthesiologists.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: The guidelines used in the service must be available.



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41.3 Anaesthetic Care

41.3.1 Standard

A pre-anaesthetic assessment is conducted and recorded.

Intent of 41.3.1

Psychiatric patients sometimes receive anaesthesia for electroconvulsive therapy. The preoperative anaesthesia assessment determines if the patient is a suitable candidate for the planned anaesthetic. The clinical assessment and results of investigations must be available to the doctor performing the assessment.

Because anaesthesia carries a high level of risk, its administration must be carefully planned. The patient's pre-anaesthetic assessment is the basis for that plan. The pre-anaesthetic assessment provides information required to:

- Select the anaesthesia and plan anaesthesia care
- Identify any drug sensitivities
- Safely administer an appropriate anaesthetic
- Interpret findings of patient monitoring

An anaesthesiologist or other qualified individual conducts the pre-anaesthetic assessment. Anaesthetic care is carefully planned and documented in the anaesthetic record. The plan considers information from the assessment of other health professionals in the multi-disciplinary team and identifies the anaesthetic to be used, the method of administration, other medication and fluids, monitoring procedures, and anticipated post-anaesthetic care.

The anaesthetic planning process includes education of the patient and his or her family or decision-maker on the risks, potential complications and options related to the planned anaesthetic and postoperative analgesia. This discussion occurs as part of the process to obtain consent for anaesthesia. The anaesthesiologist or the qualified individual who will administer anaesthesia provides this education.

41.3.1 Criteria

41.3.1.1	Patients have an anaesthetic assessment performed before the administration of anaesthesia by a qualified health professional.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Organisational policy must define who may perform this assessment and what the pre-anaesthetic assessment should include. The results of this assessment must be recorded by the individual who performs the assessment. Because the criterion refers to a medical assessment, it is scored NC if the patient record does not reflect notes by the person responsible for administering the anaesthetic. If only the nurses' assessment notes are available in the patient record, this criterion will be scored NC. Compliance will be verified during patient record audit.		

41.3.1.2	The medical assessment of patients for ECT is documented before anaesthesia.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Organisational policy must define who may perform this assessment and what the medical assessment should include. The results of this assessment must be recorded by the individual who performs the assessment. Because the criterion refers to a medical assessment, it is scored NC if the patient record does not reflect notes by a medical practitioner. If only the nurses' assessment notes are available in the patient record, this criterion will be scored NC. Compliance will be verified during patient record audit.		



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41.3.1.3	The anaesthesia assessment determines if the patient is an appropriate candidate for the planned anaesthesia.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

41.3.1.4	The name of the medical practitioner responsible for the procedure is documented.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit.		

41.3.1.5	Patients have the results of diagnostic tests recorded before anaesthesia.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit.		

41.3.1.6	The anaesthesia assessment identifies any medication allergies/sensitivities.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

41.3.1.7	Patients are re-evaluated before the induction of anaesthesia.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

41.3.1.8	The anaesthesia care of each patient is planned and documented.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		



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41.3.1.9	The patient, family and decision-makers are educated on the risks, potential complications and options of anaesthesia.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

41.3.1.10	The anaesthesiologist or other qualified individual provides the education.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

41.3.2 Standard

Each patient's physiological status is monitored during anaesthesia and recorded.

Intent of 41.3.2

The anaesthetist monitors and records the physiological status of the patient during anaesthesia and enters the drugs and intravenous fluids used and the anaesthetic in the patient's anaesthetic record. The anaesthetist has access to the patient care notes and is familiarised with the findings of the medical examination. It is important that each health professional has access to the records of other care providers, in accordance with organisation policy.

41.3.2 Criteria

41.3.2.1	The patient's physiological status is continuously monitored during anaesthesia.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

41.3.2.2	The results of monitoring are entered into the patient's record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

41.3.2.3	The anaesthesia used is entered into the patient's anaesthesia record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		



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Guideline statement: Compliance will be verified during the patient record audit.

41.3.2.4	Patient care notes are available to the anaesthesiologist.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: Compliance will be verified during the patient record audit.

41.3.3 Standard

There is a system to monitor and document each patient's post-anaesthetic status and to discharge the patient from the recovery area according to accepted guidelines.

Intent of 41.3.3

Physiological monitoring provides reliable information about the patient's status during the administration of anaesthesia and the recovery period. Monitoring methods depend on the patient's pre-anaesthetic status, choice of anaesthetic and complexity of the procedure performed during anaesthesia. In all cases however, the monitoring process is continuous and the results are entered into the patient's record.

Monitoring during anaesthesia provides the basis for monitoring during the post-anaesthetic recovery period. The ongoing, systematic collection and analysis of data on the patient's status in recovery may support decisions about moving the patient to other settings and less intensive services. Only a suitably qualified and experienced registered nurse or designated medical personnel may carry out monitoring in the recovery area. Recording of monitoring data provides the documentation to support discharge decisions.

The anaesthetist or other qualified individual decides whether the patient can be discharged from the recovery area to another level of care or from the organisation (as in the case of ambulatory anaesthesia). Standardised criteria developed by medical personnel are used to make discharge decisions. The decision to discharge the patient from the recovery area is recorded in the patient's record. The time of arrival and discharge from the recovery area are recorded. Signatures of those who handed over and those who received the patient are recorded.

41.3.3 Criteria

41.3.3.1	The anaesthetist is responsible for supervising the recovery period.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: Compliance will be verified during the patient record audit.

41.3.3.2	Patients receive monitoring appropriate to their condition during the post- anaesthesia recovery period.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		



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Guideline statement: Compliance will be verified during the patient record audit.

41.3.3.3	The qualifications and experience of personnel members who may monitor patients are documented.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: These requirements should be documented in the personnel requirements section of the job description as part of the recruitment process.

41.3.3.4	Monitoring findings are entered into the patient's record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: Compliance will be verified during the patient record audit.

41.3.3.5	The patient's physiological status is monitored during the post-procedural period.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: This includes monitoring in the recovery room and in the ward following transfer. Compliance will be verified during patient record audit.

41.3.3.6	Established criteria are used to make discharge decisions from the recovery room.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: The criteria may be documented in a policy/procedure or printed on the recovery room section of the intraoperative record. A documented record of their application will be sought during the patient record audit. The policy must state clearly who assesses the fitness of the patient to leave the recovery area and what criteria are applied.

41.3.3.7 CRITICAL	The anaesthetist applies the criteria and discharges the patient.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: Compliance will be assessed during the patient record audit.



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41.3.3.8 CRITICAL	The anaesthetist signs the patient out of the recovery area.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be assessed during the patient record audit.		

41.3.3.9	Recovery area arrival and discharge times are recorded.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be assessed during the patient record audit.		

41.3.3.10	Signatures of those handing over and those receiving the patient are recorded.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be assessed during the patient record audit.		



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41.4 Facilities, Equipment, Supplies and Medication

41.4.1 Standard

Facilities for safe surgical and anaesthetic care are provided and maintained.

Intent of 41.4.1

The design of the ECT treatment area provides space for the reception, anaesthesia, treatment, recovery and observation of patients.

There are areas for the disposal and collection of used equipment and waste, including contaminated waste and sharps. Safe and adequate storage space for pharmaceutical and surgical supplies is available including separate lockable cupboards for controlled medication.

Personnel are provided with office facilities or a day station, a restroom, washrooms and toilets.

41.4.1 Criteria

41.4.1.1	The design of the ECT treatment area provides space for the reception, anaesthesia, treatment, recovery and observation of patients.	Comment:
Severity: Serious		
Category: Structure		
NA NC PC C		
Guideline statement: This requirement will be assessed by comparing the maximum number of ECT treatment rooms in use at any one time with the number of bed spaces available for reception, anaesthetic induction, recovery and observation of patients.		

41.4.1.2	There is safe and adequate storage space for pharmaceutical and surgical supplies.	Comment:
Severity: Serious		
Category: Structure		
NA NC PC C		
Guideline statement: Control of access to pharmaceutical supplies must be evident. Storage space is often at a premium. Equipment storage may overflow into corridors. Make sure that health and safety requirements are not compromised, especially with regard to blocking escape routes.		

41.4.1.3	Access to the ECT treatment room is controlled.	Comment:
Severity: Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Only those with appropriate training should be allowed into the ECT treatment room. Patients are extremely vulnerable while under anaesthesia and while recovering from anaesthesia and should therefore be offered adequate protection. For these reasons, theatre personnel should control who is permitted to enter the theatre suite and should be aware of who is present in the treatment room at all times.		

41.4.1.4	There is access to disinfection facilities.	Comment:
Severity: Serious		
Category: Structure		
NA NC PC C		



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Guideline statement: There may not be a sterilisation unit in the ECT treatment room. There must however be access to the required sterile supplies within the treatment area.

41.4.1.5 CRITICAL	Where resuscitation or critical monitoring equipment is used that does not have built-in battery backup units, there is an uninterruptible power supply (UPS) that complies with relevant requirements and regularly serviced and tested.	Comment:
Severity: Very Serious		
Category: Structure		
NA NC PC C		

Guideline statement: This will be assessed in the ECT treatment room. It may be necessary to check with the maintenance manager regarding the servicing and testing of the UPS.

41.4.2 Standard

Anaesthetic equipment, supplies and medications used comply with the recommendations of local/international anaesthetic professional organisations or alternate authoritative sources.

Intent of 41.4.2

Anaesthetic risks are significantly reduced when appropriate and well-functioning equipment is used to administer anaesthesia and monitor the patient. Adequate supplies and medication must also be available for planned use and emergency situations. Each organisation understands the required or recommended equipment, supplies and medications necessary to provide anaesthesia services to its patient population. Recommendations on equipment, supplies and medication can come from a government agency, national or international anaesthetic professional guidelines or other authoritative sources. There is an equipment maintenance programme.

41.4.2 Criteria

41.4.2.1	The provision and use of anaesthetic mixture components complies with the guidelines for practice of the professional society.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: Organisation policy should reflect the requirements of the country's professional body or another authoritative source.

41.4.2.2	The provision and use of breathing circuits complies with the guidelines for practice of the professional society.	Comment:
Severity: Very Serious		
Category: Structure		
NA NC PC C		

Guideline statement: An assessment will be made whether the most recent copy of the guidelines of the country's professional body or a copy of another authoritative source is available and is used to guide the choice of anaesthetic apparatus.



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41.4.2.3	The provision and use of ancillary equipment complies with the guidelines for practice of the professional society.	Comment:
Severity: Very Serious		
Category: Structure		
NA NC PC C		
Guideline statement: An assessment will be made whether the most recent copy of the guidelines of the country's professional body or a copy of another authoritative source is available and is used to guide the choice of ancillary equipment.		

41.4.2.4	The provision and use of monitoring equipment complies with the guidelines for practice of the professional society.	Comment:
Severity: Very Serious		
Category: Structure		
NA NC PC C		
Guideline statement: An assessment will be made whether the most recent copy of the guidelines of the country's professional body or a copy of another authoritative source is available and is used to guide the provision of monitoring equipment.		

41.4.2.5	Medication is available according to the requirements of the level of care provided.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Either the guidelines of the country's professional body or the national guideline on essential drugs can be used as the basis for deciding which medication should be available. Where such guidance is not available, the guidance from surrounding countries or international bodies should be consulted.		

41.4.2.6	A medication trolley is available for the exclusive use of the anaesthesiologist in each theatre.	Comment:
Severity: Serious		
Category: Structure		
NA NC PC C		
Guideline statement: Each anaesthetist must have immediate access to all medication required.		

41.4.2.7	Theatre personnel ensure that all equipment is included in the organisation's equipment replacement and maintenance programme.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence is required.		

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41.4.3 Standard

Emergency and protective equipment are provided in the operating theatre.

Intent of 41.4.3

Electro-convulsive therapy personnel must prepare for any emergencies through the provision of emergency and protective equipment.

Every patient care area must have access to resuscitation equipment within one minute of any patient collapsing that includes as a minimum:

- Defibrillator or automated external defibrillator (AED) with adult and paediatric (where applicable) paddles/pads available within three minutes of any patient collapsing
- ECG monitor
- CPR board
- Suction apparatus, hard and soft suction catheters and suction tubing
- A bag-mask manual ventilator (Ambubag) or equivalent
- A range of endotracheal tubes and two laryngoscopes, with a range of straight and curved blades, spare batteries and spare globes where applicable
- An introducer/stylet for endotracheal intubation
- A syringe to inflate the endotracheal tube cuff
- A laryngeal mask airway
- Oral airways
- Equipment to perform an emergency cricothyroidotomy
- Medication for cardiac and respiratory arrest, coma, fits and states of shock (including paediatric doses where applicable)
- Appropriate consumables for intravenous therapy and medication administration and volume expanders (including paediatric sizes where applicable)

41.4.3 Criteria

41.4.3.1 CRITICAL	Emergency resuscitation equipment is available.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: As a minimum, the equipment set out in a) – m) in the standard intent above must be available.		

41.4.3.2	Emergency resuscitation equipment shows evidence of regular checking.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence of regular checking of resuscitation equipment according to organisational policy should be provided for assessment.		

41.4.3.3	There is a mechanism for summoning assistance.	Comment:
Severity: Very Serious		
Category: Structure		
NA NC PC C		



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Guideline statement: A mechanism should be in place to ensure that assistance can be summoned rapidly and effectively should the need arise. Various methods such as for example, alarm, intercom, alert button or functional telephone/pager systems are acceptable. The mechanism to be used should be documented in a policy and new team members educated regarding the content of the policy.

41.4.3.4	There is appropriate protective clothing available.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: All standard personal protective equipment should be available and in addition, personal protective equipment appropriate for the procedures offered (for example, lead aprons for orthopaedic theatres) should be available to ensure the safety and continued wellbeing of personnel.		

41.4.3.5	Emergency and resuscitation equipment and supplies have clearly defined instructions for use.	Comment:
Severity: Very Serious		
Category: Structure		
NA NC PC C		
Guideline statement: Instruction leaflets for equipment and package inserts for all medication and supplies should be available to be consulted when necessary. Where equipment or supplies requiring rapid deployment in an emergency situation are used infrequently and personnel are therefore likely to be unfamiliar with their use, it is recommended that "flashcards" are made available or information sheets are readily to hand, for example, resuscitation procedures posted on the wall in the recovery room.		

41.4.3.6	Hazard or warning notices are displayed.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: These notices should be in local languages with associated pictograms to illustrate the warning for those unable to understand the written words. They should be prominently displayed in close proximity to the potential hazard.		

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41.4.4 Standard

Recovery room facilities and equipment are available to provide safe and effective care.

Intent of 41.4.4

The number of beds/trolley spaces in the recovery room provides sufficient space for at least one patient from each ECT treatment room that it services and is sufficient for peak loads. The provision, use and maintenance of recovery room equipment comply with the guidelines for practice of local/international professional societies.

41.4.4 Criteria

41.4.4.1	The recovery area forms part of the ECT suite and there is direct access to the treatment room from the recovery area.	Comment:
Severity: Serious		
Category: Structure		
NA NC PC C		
Guideline statement: This ensures the close proximity of responsible clinicians for appropriate medical assistance for patients postoperatively is available as quickly as possible and facilitates the rapid return to the ECT room if required.		

41.4.4.2	There are an adequate number of recovery beds for the patients.	Comment:
Severity: Serious		
Category: Structure		
NA NC PC C		
Guideline statement: This requirement will be assessed by comparing the maximum number of ECT treatment rooms in use at any one time with the number of bed spaces available for recovery and observation of patients.		

41.4.4.3	There is adequate lighting.	Comment:
Severity: Serious		
Category: Structure		
NA NC PC C		
Guideline statement: Lighting should not be harsh and should comply with recommended standards. Local lighting to assist clinical examination must also be available.		

41.4.4.4	The provision, use and maintenance of recovery room equipment comply with the guidelines for practice of the relevant professional society.	Comment:
Severity: Very Serious		
Category: Structure		
NA NC PC C		
Guideline statement: Guidelines from the relevant professional society should be available and the theatre suite should comply with all the minimum requirements.		



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41.5 Quality Improvement

41.5.1 Standard

A formalised proactive quality improvement approach is maintained in the electro-convulsive therapy service.

Intent of 41.5.1

The senior management team is responsible for ensuring that standards are set throughout the organisation. Within each department or service, unit managers should ensure that standards are set for the particular unit. Departmental or service managers should use available data and information to identify priority areas for quality monitoring and improvement. This should be done in collaboration with the organisation's central quality management structures to ensure coordinated quality improvement activities throughout the organisation.

Quality monitoring is typically applied to high risk, high volume or high cost activities, or areas of concern identified by personnel, patients or visitors. Some examples of activities that may benefit from quality monitoring include:

- Patient assessment
- Procedures carried out
- The use of medication
- Medication errors
- Patient and family expectations and satisfaction

The following will be evaluated:

- The manner in which problems were identified and prioritised in this service for which quality improvement activities were initiated
- The processes put in place to resolve the problems
- The identification of indicators to measure improvement
- The tool(s) used to evaluate these indicators
- The monitoring of these indicators and corrective steps taken when goals were not achieved
- Graphed and/or tabled results, as appropriate

A once-off project such as acquiring a specific item of equipment will be scored NC. Quality improvement processes not related to the clinical quality of patient care but to the environment within which care is provided, for example, monitoring the checking of the emergency trolley over time, will be scored PC.

41.5.1 Criteria

41.5.1.1 ROOT Severity: Very Serious Category: Efficiency NA NC PC C	There is a written quality improvement programme for the electro-convulsive therapy service that is developed and agreed upon by the personnel of the service.	Comment:
Guideline statement: The minimum requirement for compliance will be the availability of evidence of: • At least one clinical audit • Participation in documentation (patient record) audits • Participation in the monitoring of near misses, adverse events and sentinel events		

41.5.1.2 Severity: Very Serious Category: Efficiency NA NC PC C	Indicators of performance are identified to evaluate the quality of treatment and patient care.	Comment:
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Guideline statement: As the emphasis is on the quality of patient care, this cannot be compliant if there is no evidence of clinical audits. These could take the form of evaluation of the implementation of clinical guidelines, case discussions, reports on negative incidents, etc. An indicator, in this context, is a measure used to determine improvements in clinical care over time. The information gained must be used to identify and implement improvement processes. Statistical reports without documented evidence of continuous quality improvement will be scored PC.

41.5.1.3	Processes are selected in order of priority for evaluation and improvement in the quality of treatment and care.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Quality monitoring is typically applied to high risk, high volume or high cost activities, or areas of concern identified by personnel, patients or visitors.

41.5.1.4	The quality improvement cycle includes the monitoring and evaluation of the standards set, and remedial action implemented.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: The quality improvement cycle does not stop with the implementation of a quality improvement plan. Ongoing monitoring of the process or procedure under consideration is required, with evaluation of the monitoring data collected to ensure that the plan has achieved its intended aim. If not, the quality improvement cycle should be repeated, incorporating the learning from previous attempts, until the desired aim is achieved.

41.5.1.5	A documentation audit system is in place.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Documented evidence of such audits must be provided at departmental level. Where the documentation (patient record) audit is an organisation-wide multidisciplinary process, which includes all clinical departments, this criterion will be scored according to the findings of the involvement of each department in the process. Evaluation of results and remedial action taken must be documented as well as maintenance of achieved improvements over time.

41.5.1.6	An incident managing system for monitoring near misses/adverse events/sentinel events is implemented, which includes the documentation of responses to recorded incidents and interventions to prevent recurrence of the incident or minimise harm in the event of a recurrence.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Participation at department level in the organisation's overall system of the monitoring of near misses/adverse events/sentinel events will be evaluated. Analysed data, with responses/remedial action related to departmental incidents are required.



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41.6 Patient Rights

41.6.1 Standard

The organisation is responsible for providing processes that support patient and family rights during care.

Intent of 41.6.1

This refers to the implementation of organisational policy on patient and family rights. Compliance will be verified during observation of patient care processes, patient record audits and patient interviews.

41.6.1 Criteria

41.6.1.1 ROOT	There are processes that support patient and family rights during care.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: The organisation's policies will be evaluated against country-specific legislation, where applicable. The implementation thereof will be evaluated during the auditing of patient records, patient interviews and observation during a walk-about in the department. This applies to all the relevant criteria below.		

41.6.1.2	There are processes to ensure that care is respectful of the patient's personal values and beliefs.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Implementation will be evaluated during the auditing of patient records, patient interviews and observation during a walk-about in the department.		

41.6.1.3	Measures are taken to protect the patient's privacy, person and possessions.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Confidentiality of patient information and patient privacy must be protected at all times. Standard operating procedures specific to the conditions and processes in the unit must be in place, known to relevant personnel and implemented. Likewise, security measures specific to the unit must be in place to safeguard the patients' person and possessions.		

41.6.1.4	Personnel respect the rights of patients and families to treatment and to refuse treatment.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit and patient interviews.		



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41.6.1.5	Patients are informed of their right not to donate human tissue or to participate in research.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: It is very important to ensure that this internationally accepted right is implemented in the mental health patient population.		

41.7 Prevention and Control of Infection

41.7.1 Standard

There is a comprehensive infection prevention and control programme encompassing patient care and employee health.

Intent of 41.7.1

This standard measures the implementation of the organisational policy for infection prevention and control at unit level as well as the identification of unit-specific infection control risks. Personnel must be aware of the policy and there must be evidence that it is implemented throughout the organisation.

41.7.1 Criteria

41.7.1.1 ROOT	There is a programme, which is implemented, to reduce the risks of healthcare associated infections in patients and healthcare workers.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: • The programme must include as a minimum: • Prevention of the spread of respiratory tract infections • Prevention of the spread of urinary catheter related infections • Prevention of the spread of infection through surgical wounds • Prevention of the spread of infection through intravascular/invasive devices • Safe injection practices, including single-use injection devices • Care of the infected patient • The wearing of personal protective equipment • The disinfection of equipment • Management of infected/soiled linen • Management of hazardous waste		

41.7.1.2	The department participates in the overall programme for quality management and improvement of infection control.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Compliance will be measured by evidence of departmental representation at organisational infection control meetings and activities and by the departmental contribution of data to the organisational infection control data collection systems.		



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41.7.1.3	The department identifies the procedures and processes associated with the risk of infection, and implements strategies to reduce risk.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: The department participates in, and has documented evidence of, the identification of risks in their department. The strategies to reduce risk should be defined in policies and procedures and include the risks listed in this standard as a minimum. Such documentation must form part of the organisation-wide infection control processes.

41.7.1.4	Personnel are trained in correct hand washing procedures.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: Documented evidence of personnel training will be required, together with evidence of monitoring of implementation, for example, hand washing audits performed regularly according to organisational policy.

41.7.1.5 Organisational policy on handling, storage and disposal of healthcare waste is implemented.

Guideline Statement:

The implementation of the organisation's master policy will be evaluated in the department. The processes must include handling at source, collection, storage and disposal of healthcare waste.

41.7.1.6 Individuals who collect specimens are trained in the proper collection and handling of microbiological specimens.

Guideline Statement:

Documented evidence of personnel training will be required.

41.7.1.7 The department provides education on infection control practices to personnel, doctors, patients and, as appropriate, family and other caregivers.

Guideline Statement:

Documented evidence of personnel training will be required. Information pamphlets, posters, individualised education etc. will provide evidence of patient, family and other caregiver education

41.7.1.5	Organisational policy on handling, storage and disposal of healthcare waste is implemented.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: The implementation of the organisation's master policy will be evaluated in the department. The processes must include handling at source, collection, storage and disposal of healthcare waste.

41.7.1.6	Individuals who collect specimens are trained in the proper collection and handling of microbiological specimens.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		



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Guideline statement: Documented evidence of personnel training will be required.

41.7.1.7	The department provides education on infection control practices to personnel, doctors, patients and, as appropriate, family and other caregivers.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence of personnel training will be required. Information pamphlets, posters, individualised education etc. will provide evidence of patient, family and other caregiver education		

41.8 Occupational Health and Safety and Risk Management

41.8.1 Standard

The organisation directs and controls risk management programmes, which include health and safety programmes that comply with legislation.

Intent of 41.8.1

This standard measures the implementation of the organisational policy for occupational health and safety and risk management at unit level as well as the identification of unit-specific risks. Personnel must be aware of the policy and there must be evidence that it is implemented throughout the organisation.

41.8.1 Criteria

41.8.1.1	The department conducts ongoing monitoring of risks through documented assessments as part of organisation's risk management processes.	Comment:
ROOT		
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: The department participates in and has documented evidence of the identification of risks in their department. Such documentation must form part of the organisation-wide risk management processes. All relevant aspects and services of the department, for example, patient, personnel and visitor related risks, financial and legal risks, physical facilities, security and environmental risks, etc. should be included.		

41.8.1.2	The Health and Safety representative for the department supervises implementation of the organisation's health and safety programme.	Comment:
Severity: Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence is required.		



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41.8.1.3	Fire safety measures are implemented.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: The department's participation in the organisation's overall fire safety processes will be evaluated.		

41.8.1.4	Relevant personnel are trained in the procedures relating to the reporting and investigation of near misses/adverse events/sentinel events.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence of training required.		

41.8.1.5	Security measures are implemented to ensure the safety of patients, personnel and visitors.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: The department's participation in the organisation's overall security plan for personnel and visitors will be evaluated.		

41.8.1.6	All personnel are trained regarding their role in providing a safe and secure patient care facility.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: This is of particular relevance to the mental health care setting where all personnel must be alert to the safety and security of vulnerable patients. Documented evidence of training required.		